**MERIDIAN BAY MEDICAL CENTER**

Internal Medicine — Inpatient Service

**DISCHARGE SUMMARY**

**FICTIONAL DOCUMENT — FOR SOFTWARE TESTING ONLY. All patient details, drug names, doses, and clinical content in this document are entirely fictional. Nothing in this document represents real medical guidance and it must not be used for any clinical purpose.**

**Patient:** Daniel Okonkwo, 58-year-old male

**MRN:** MBMC-7714209

**Admission date:** March 4, 2026

**Discharge date:** March 11, 2026

**Attending:** Dr. R. Nakashima, MD

**Service:** Internal Medicine

## 1. Reason for Admission

Mr. Okonkwo, a 58-year-old man with a past history significant for borderline hypertension and dyslipidemia, presented to the emergency department on March 4 with a five-day history of progressive dyspnea on exertion, intermittent palpitations, and one episode of chest tightness lasting approximately twenty minutes that resolved spontaneously. Initial blood pressure on presentation was 188/104 mmHg, heart rate was 112 beats per minute and irregular, and oxygen saturation was 96% on room air. Initial ECG demonstrated atrial fibrillation with rapid ventricular response. Troponin was mildly elevated on first measurement and trended down on serial draws; the patient was ruled out for acute coronary syndrome.

## 2. Hospital Course

The patient was admitted to the internal medicine service for management of newly diagnosed atrial fibrillation, hypertensive urgency, and ongoing dyslipidemia. Rate control was achieved with intravenous medication on day one, then transitioned to oral therapy on day two. Blood pressure was titrated downward over the course of the admission, reaching a stable range of 128–134 / 78–84 mmHg by day five. The patient also reported worsening anxiety in association with his new diagnoses, which was discussed with the inpatient psychiatry consultation service; a short course of anxiolytic therapy was recommended.

By day seven the patient was clinically stable, hemodynamically controlled, and ready for discharge. Outpatient follow-up was arranged with cardiology and primary care. Patient counseling was provided on each of the discharge medications, with particular emphasis on medication adherence and on contacting the clinic for any unusual side effects.

## 3. Discharge Diagnoses

1. New-onset atrial fibrillation, rate-controlled.

2. Hypertensive urgency, resolved.

3. Hyperlipidemia.

4. Acute anxiety, situational, related to new cardiovascular diagnoses.

5. GERD, mild — prophylaxis given concurrent medication burden.

## 4. Discharge Medications

The following regimen was prescribed at discharge. The patient was counseled on each medication, its purpose, and key warnings. A printed medication list was provided and reviewed with the patient and his spouse at the bedside prior to discharge.

| Medication | Dose | Route / Frequency | Duration | Indication |
|---|---|---|---|---|
| Velantine | 10 mg | PO once daily | Continue indefinitely | Hypertension |
| Cordizem-XR | 100 mg | PO once daily | Continue indefinitely | Rate control, hypertension |
| Pravoxil | 20 mg | PO every morning | Continue indefinitely | Hyperlipidemia |
| Etrazolam | 0.5 mg | PO twice daily | 8 weeks | Acute anxiety, sleep disturbance |
| Pranixol | 10 mg | PO once daily | Continue indefinitely | Adjunctive cardiovascular protection |
| Quintaxin | 20 mg | PO once daily, before breakfast | 8 weeks, reassess | GERD prophylaxis |

**Notes on regimen:** Velantine was selected as first-line antihypertensive given the patient's good renal function and absence of contraindications. Cordizem-XR was added for rate control of the atrial fibrillation. Pravoxil was prescribed for hyperlipidemia management given the patient's elevated LDL on admission labs. Etrazolam was prescribed at the recommendation of the psychiatry consultation service for the patient's situational anxiety; the patient was counseled that this medication is intended for short-term use and was given a follow-up appointment with outpatient psychiatry in two weeks for reassessment. Pranixol was added per cardiology recommendation for adjunctive cardiovascular

protection. Quintaxin was added for GERD prophylaxis given the multi-drug regimen and the patient's reported intermittent heartburn.

## 5. Counseling and Warnings Reviewed with Patient

The following points were reviewed with the patient and his spouse prior to discharge:

• Velantine may cause a persistent dry cough; the patient should report this if it develops. The patient was advised that the medication is contraindicated in pregnancy, which is not applicable in his case but was noted for completeness.

• Cordizem-XR may cause fatigue, particularly in the first one to two weeks. The patient was counseled to take the medication consistently in the morning and not to discontinue abruptly.

• Pravoxil should be taken in the morning with breakfast for best tolerability. The patient was advised to report any unexplained muscle pain or weakness immediately, as this may indicate a serious side effect.

• Etrazolam should be taken twice daily as prescribed. The patient was counseled that this medication may cause drowsiness and that he should not drive or operate heavy machinery until he understands how the medication affects him. The 8-week course was discussed with the patient, with outpatient psychiatry follow-up scheduled at the 8-week mark to evaluate response and consider continuation.

• Pranixol should be taken at the same time each day. No specific dietary restrictions were noted.

• Quintaxin should be taken thirty minutes before breakfast for optimal effect.

## 6. Follow-Up

The patient was instructed to follow up with primary care within seven days of discharge, with cardiology within two weeks, and with outpatient psychiatry within two weeks for medication reassessment. A repeat lipid panel was ordered for six weeks post-discharge to assess response to Pravoxil. A basic metabolic panel was ordered for two weeks post-discharge to assess renal function on the new antihypertensive regimen.

The patient verbalized understanding of the discharge instructions, medication regimen, and follow-up plan. He was discharged home in stable condition in the company of his spouse.

*Dictated and signed: Dr. R. Nakashima, MD — March 11, 2026 — 14:22 PST*